

I: So then we're gonna start with the actual questions. This one is about familiarity with AI. How familiar are you with AI technologies in general?

P: I mean, I'm just a user of AI tools, especially to search. Just similar to how we use Google. So we search for something. The difference is Copilot. So if you ask for something, it can create that more customized to what you really need, like scientific articles, or restructuring some texts that you want to use to communicate in social media. More like guides, not to do the job, let's say.

I: Could you also give a score of one to ten, how familiar you think you are? One being low and ten being high.

P: I don't think I am really familiar. I only use ChatGPT and the Copilot is already installed in our computer. So I would say a four.

I: So you said you're using ChatGPT and also Copilot. You do freelance, so you work with different companies, right. Do you use Copilot in all of those organizations or is it one company allows you to use it and one company doesn't?

P: I don't use Copilot or AI tools in my practice. I use it as a tool to communicate in social media, or to make a research for my study. I don't use AI on my work.

I: So you don't use it in treatment, but you do use it for, for example, your own learning. You're not in university, but you're still learning more every day, right?

P: Yeah, exactly. In my study, in going deeper into my knowledge, I'm remembering that I use ChatGPT in one context specifically. It's about searching for concepts, the meaning of some concept, because in some exercises we ask the person to know what means loyalty or whatever. And sometimes people don't know how to do that, they don't know the meaning. So we asked Copilot for this concept and then we work from this definition. But it's kind of in a way of a joke, because sometimes people bring us this behavior of, I also asked ChatGPT but he didn't say much. So let's ask ChatGPT what this means. And sometimes I use it in that context.

I: So you have used it in that context, and in that case it was mostly like a definition tool, right. Is it that they didn't understand the term in Portuguese or was it in English?

P: No, even in Portuguese. For example, what means loyal? What is loyalty? Or what does it mean being honest? Or what's the difference between forgiving and forgetting?

I: And then you would just ask ChatGPT that or Copilot and it would just give some...

P: I invite people to ask ChatGPT and then we discuss if the definition makes sense for the person.

I: So your organization itself has not introduced any AI tools formally. And in those cases that you did use AI, you did tell them that you were using AI, right? Because you kind of use AI with them, so they already know.

P: Yeah, exactly.

I: What are your thoughts on AI being used in diagnostic or assessment processes?

P: It's a tricky question, because in what I feel, even talking with other colleagues in the beginning of their careers, for example, it can be too easy to ask ChatGPT to interpret some results of clinical evaluations, or to build this conceptual model for interventions. And in psychology, at least, each intervention plan is unique. It should be unique. You can have different sources, different models of intervention, different techniques and you make a combination of all of that and you put them in some order that you will use according to the person that you have, according to the needs that this person is bringing in that moment. So it's very specific. Also, there is this thing that I have noticed, that ChatGPT, it's the only platform that I know until now, it's more pretentious, like it goes in order to confirm your beliefs or to confirm what you are bringing. And not only this, it doesn't work in a therapeutic process. There is something in a therapeutic process that we need to pay attention to, that it's about whether the way the person is talking about the situation is actually what they are feeling about the situation. So you have this mismatch between what they say and what they are doing or what they are expressing. And this divergence is not the same in everyone. You need to know the person to see what micro-expressions can be activated in their faces or in their bodies, combined with what the person is saying. So it's this kind of detail. I confess that I already forgot the question.

I: That's okay. So the question was, what are your thoughts on AI being used in diagnostic or assessment processes?

P: Okay, so all of these to say: I think it can be a research tool to check on some details, to check on some differential diagnosis, in the sense of checking if this symptom is common in other pathologies, but never to confirm the diagnosis.

I: So to summarize, there is some kind of mismatch between what people are saying and what they're actually feeling, right. So sometimes there might be some more qualitative data, so for example, someone is touching their face or whatever is happening, and AI cannot really see that. So there is a mismatch between the data that AI will get and the data that a person might get.

P: Right.

I: And then also you mentioned that it might jump to conclusions and it also has this kind of confirmation bias. So if you give it some information, it might tell you, oh, it's this, or you're totally right, it is anxiety disorder or whatever. But maybe if you look at it on a personal level, there might be some deeper cause for some anxious symptoms or whatever, right. And maybe as a person, you would not jump to that conclusion that fast, but AI tends to be like, oh, it's anxiety disorder, right?

P: Yeah, I can give you examples. For example, in some personality traits, personality patterns, you can have anxiety symptoms or depression symptoms or even avoiding symptoms, and this doesn't mean that the person has depression or has anxiety or has a problem in their relationships. There is a context where these symptoms appear and there are triggers in these contexts that for you can be a trigger for some symptom and for me can be another one. So it's so specific. There is a joke, a general joke, I don't know if it's a global joke or just in Portugal, that psychologists, when you ask something to a psychologist, they always answer you with, it depends, because it is really specific in each case.

I: So it's very context dependent. So for example, for you a trigger might be whatever, and then for me the same trigger or the same external motivators wouldn't result in anything. I wanted to check one more thing. You said you could use it as a research tool and to do differential diagnosis, but you said not to confirm a diagnosis. Do you mean that you shouldn't ask ChatGPT or Copilot, oh this is my diagnosis, is this correct? Do you think that's a wrong use?

P: I think it's a wrong use. The same way when we ask Dr. Google, for example. Also, because of this thing that I told you before, these pretensions, they tend to confirm, to follow your thoughts, to validate your thoughts. And not all thoughts should be validated. So, according to the diagnosis, it's usual that people get a diagnosis and then they will search for what does it mean, what this means in relationships, in work, in whatever. Just to understand better. It's not only the best option to share the diagnosis with a person. You should share, but the elements that confirm the diagnosis go far beyond the criteria of the DSM, for example. You can have this checklist in the manual and you have all of that and still it's not a disorder. Because of the context, because of what motivates these symptoms or how long they have had these symptoms and whatever.

I: Yeah, so the DSM has a lot of criteria for specific disorders. So you're saying that sometimes maybe someone ticks a few boxes, but that doesn't mean necessarily that that is a disorder, because it might not be a problem in their life or not a problem for anyone around them. So it's not necessarily a problem that should be addressed, right. Like maybe there are some symptoms that you want to work on, but it doesn't necessarily mean that someone has a depression or that there is a very strict way that we need to diagnose them. So can I also assume that for intake processes and preliminary screening, you also don't think AI would be useful there?

P: I don't know if I understand the question.

I: So the intake, I think you're doing intakes as well with your clients, right? The first session with someone is an intake, you ask them a lot of questions and you try, maybe not necessarily to diagnose, but to figure out what's going on. And also maybe even before the intake, you might do screening, so give them a questionnaire or ask them questions beforehand. You also think that AI cannot be useful there in those areas?

P: It could be useful in the way that AI has an application or a program that we can use to process the data. In this kind of questionnaires it can be really useful, but it cannot be the one

which will interpret the data. It also cannot be the one who will search for this anamnesis, this background from the person, this intake. Because according to what the person says, you can follow your interview, your structured interview, or you can go around, or you can notice something that triggers you and you go from this way to this way. But also in this, could you repeat the question? Because I would like to say something more.

I: So I added two questions, which is what about the intake processes and have you considered AI for preliminary screening? So the screening beforehand or during the intake, do you think AI could be used for that?

P: So when we use these structural evaluations like questionnaires, not all the approaches use this kind of tool. And not always we use this in the beginning of the process. Not in every process we use the same questionnaires. And what I would like to say more about this is how dangerous it can be for a person to know their results without context. So for AI platforms having access to these kind of tools can be really dangerous. These tools are used in a setting, which means we use them only in session and it's confidential, and you cannot share with people outside because of the confidentiality. So how could we guarantee that there will not be a bug that shares this with people or a hacker that has access to that? In the computer, when we make our registrations, our conceptual models, etc., we never work with the name of the person because of these hackers and things.

I: So currently in your job, if you make any report or whatever, you also try and not include that much personal information for data privacy.

P: Yeah, data privacy. And all the information that we produce, for example, to refer to a psychiatrist or to send to a school, to a professor, all the content in the document has client consent.

I: So how do you currently have the connection between the actual client and the documents? Because if you cannot use a name, is there some identifying number or how is this connected?

P: A code.

I: So there's just some place in your organization that has their names and some code and that code corresponds with a report?

P: It can be initials, like Person One, P1. for example. But they are not in the clinic. They are with the therapist. I'm the one who is responsible for the data. So I am the one who creates the codes.

I: So it's not like 100% safe, right? Because maybe if someone steals from you, then they will have that data. But there are a lot of steps or a lot of things that you put in place, so it's very unlikely that all of those steps would be compromised, because even if someone gets the data from you it doesn't mean that they can necessarily identify who this person is.

P: And this is useful even in supervision. We have supervision. It's a meeting that we have with other colleagues to get inputs on an intervention.

I: Like a second opinion, right?

P: Like a second opinion. And even in those cases where confidentiality is guaranteed between us, we still don't share the name or the personal details.

I: How do you view the role of AI in tracking patient progress over time?

P: I mean, there are some apps that already make this monitoring of mood, some exercises. But I cannot imagine how AI could monitor symptoms or achievements in the therapeutic process. Maybe because I don't know enough about AI, but I cannot imagine a way.

I: I don't want to steer you in any way, but maybe if you could explain a bit about how monitoring is done currently. So do you do any monitoring of your clients, or how do you track if they're going the correct way or if their symptoms are getting worse?

P: Yeah, we have this behavioral observation, which means that in each session we check on the symptoms. And there are different kinds of symptoms. There are these emotional symptoms that people currently know about, anxiety for example. We can check on how many panic attacks, how many difficulties regulating the breathing, etc. But there are also other kinds of symptoms. So according to the map that we build, for example, in the beginning of the process, of what kinds of symptoms are typical in this person when they experience anxiety or sadness, we can check how often they have these symptoms in this week or this month, or how effective was the tool that we have to help regulate the breathing, for example, or how is their sleep. And how many times, for example in OCD, how often do you check on things, how often do you have this need to clean things. So we can make an app where we can create these kinds of schedules. But honestly, I don't know if the app is integrated with AI, because people already did apps before AI, so I think it's not the same.

I: So as I understand it, in the beginning of the treatment you identify some symptoms and you track these over time, so every session you make observations. And there are some very factual observations, so maybe, oh, I slept this many hours, or whatever. But there are also some more subjective observations. So for example, if your client would say, oh, I'm feeling really sad, there's not a number for really sad, right. But there's still an observation for that. So you could mention it in some tracking or in a report, you just write it down, feeling more sadness this week. And maybe there's some trigger for this. But currently what you're doing is you do the sessions with your clients and then during the sessions you write down, oh, they mentioned they slept badly or they are feeling worse.

P: Yeah. And usually we correlate this to some episode or something that the person is dealing with. You were talking and I was remembering. I forget.

I: Remembering and then forgetting.

P: Yeah. I was thinking to say something, because you were talking about this monitoring. I don't remember.

I: That's fine. So how would you feel about AI taking on monitoring tasks in your practice? You kind of don't feel like there is much space for this, right?

P: Yeah, I cannot imagine a way that AI could help us monitor that, that is not already done in other platforms.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: Scheduling reminders, you said. I think this was the thing that I was thinking about that I forgot and now I think I remember again. About the scheduling, it could be nice for small entrepreneurs to have this assistant in scheduling, but I'm not sure if this already exists, like a Nona or a book or, yeah. Can you repeat the question just to check if I forgot something?

I: So the beginning of the question is, how do you feel about AI generating drafts or reports? And then the rest was, or assisting with scheduling and reminders. The scheduling and reminders you answered, but do you think there's any place for AI to be summarizing sessions or making drafts or reports?

P: I can only imagine about reports, for example, maybe helping to organize information. But usually people build their own report structure. I mean, this is an example that could work as a guide or suggestion tool. I would like to report these results, how should I communicate them? And it finds some examples of reports and says, usually this kind of results are reported like that. But this is a help that I only imagine being useful for beginners. I feel that my suggestions are mostly related to study, help in our professional context.

I: For most of the things you're saying, the actual idea of AI taking over tasks is not really relevant or possible, but it might be able to support in some kind of way. So for example, with the report writing, it shouldn't take over the actual report writing, but it might be useful to use it and ask, hey, how should I be writing this report? And that is also more useful for people that are just starting out. Someone that has more experience already has their own technique or their own way of writing a report, so they won't really need it.

P: For example, I already have my report structure, my notes structure, the way I do it. And it's changeable in the sense that if I do some course or some training, I can learn something more and it can make sense for me to add a section in my report structure. There is a big challenge in our profession, because it's so customized, our work, it's really difficult to have access to examples or to check, okay, to this we do that, to this we do that. We cannot do this in our profession. So in the beginning it can be really hard to feel confident about some communication documents. That's why I can see how useful it could be in these kinds of situations. But there is always this but: you cannot consider this for everything. It's just a guideline. It's just a suggestion.

I: So it might be useful to come up with some suggestions or some ideas, but it could also be used to try and be a bit creative. Maybe it gives me this layout or this structure and I take this part and that is useful for me. But then again, also because psychology is so very context dependent and person dependent, it's dependent on the client and also on the psychologist. Everyone has their own style. And I think what research shows is that the most important thing is how the connection is between client and psychologist.

P: That's what shows the benefit of therapy, right. And not necessarily how therapy goes. It's mostly the connection. So it's about relationships, and everything is about relationship, and the relationship is the base where everything happens.

I: What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I cannot let go of this thought that somehow AI can be an upgrade of Dr. Google, and it can be really useful if the sources are real and correct. But I have experienced with other colleagues that this psychoeducation that AI does is not always correct and really useful. So I'm really skeptical of how much risk this possibility has between sessions. It could be nice to create tools, and this can be a difference from other platforms or other digital sources, to create documents. Like, okay, I would like to make a table to check some kind of thoughts or to monitor some kind of symptoms, and the AI produces your document. So you don't have to go to Word or Excel or things. But again, for this kind of layout, I use Canva. So I'm not sure if Canva is AI. Maybe I'm not a four, maybe I'm a three.

I: I just had a discussion with my supervisor about this. The goal of the study is just to figure out how familiar psychologists think they are, and not how familiar they think they are after we discuss a lot of things, right. So I will keep the score the same, but now you at least understand why. So what you're saying is you don't really see that many opportunities for psychoeducation and between session tools, right. And you also don't think it would be beneficial for clients to have some AI tool that they could interact with or use between the sessions they have with their psychologist?

P: No, I don't think so. I cannot imagine a way that this could be helpful.

I: What risks or concerns do you have about using AI in clinical work?

P: The risks I'm concerned about are using AI freely to discuss our emotional things, issues. It's the perception that people build of how shallow this comprehension can be. It's been really often that people appear with some rational thought of why do I avoid romantic relationships, because my dad was that, because my dad did something. And this can not only be wrong, but also compromise the therapeutic process, because people build some belief around this thought and it can put them more into the problem instead of giving them a real comprehension. You know, people connect with this kind of impression. Even this week, I have this girl who said, I know I have daddy issues. And how do you know that? TikTok. So how does TikTok pretend to solve that? And why do you think you have daddy issues? Because that, because this, something was

right, something was wrong, but it's not always a good input on the therapeutic process. Also because therapy is not a place just to understand why. It's more, why not in a different way? Like, why do I feel like that? It's more, why not feeling in another way? What's stopping me from feeling differently in a positive way. So it doesn't matter for you to know you have daddy issues if this doesn't solve the relationship you have with your dad. And poor dads, maybe he's right and he's just being in a wall of shame because he did some mistake. So this is one of my concerns. The other limitation we already shared in our conversation, it's about the relationship, this therapeutic alliance. Not all the therapists will work with all people. I can be your therapist and we work together, or maybe you don't like the way I work and then you go to another person. It can be something that's really personal for the person. It's being too general and not making the person feel special the way they are. It takes from the client their identity.

I: Um, anything else?

P: No, I think it's good.

I: So I wanted to clarify a few things. You said that sometimes people come in with some beliefs that were formed in some way, maybe from TikTok or from somewhere else, that are maybe blocking their progress. Because with this belief they come to an understanding, oh I have ADHD or I have anxiety disorder or I have daddy issues or whatever. And this might block them from actually getting into a normal life where they might try and fix these issues. So you said it's not about the why, but it's about the why not. So as I understand it, what you mean is that sometimes people come up with their beliefs, but these beliefs are not always helpful in getting to a better position, right?

P: Yeah, because sometimes these beliefs that are seen on an AI platform or in digital sources, they are validating primary beliefs. For example, I think my father, I think men cannot love me. And that's because you have daddy issues. This information doesn't solve your problem. So it puts even more weight on your primary belief. So it's this tendency to validate beliefs that may not be the best for the person.

I: So it's sometimes good to have validation, right. And say, okay, this is correct, or it's valid that you have this feeling, or this might be the cause of your beliefs, right. And sometimes AI is correct in that way, where the person says I have daddy issues and this and this and this, there might be some things that are correct. But there might also be other things that might not be correct. And also sometimes it's not important to look at, okay, these are the reasons or these are the things that are causing this, but maybe look at, okay, but how do you fix that? And also sometimes you need to point out, hey, but maybe this one thing that you mentioned is unrelated or is not correct, and therefore let's not confirm it, right. Then we need to dispute that. Where maybe AI will say, oh yeah, that's because of this or because you had this problem when you were five years old or whatever, instead sometimes a psychologist's task is to say, hey, how about maybe you did something wrong there, or maybe something else happened, or there's always different options, right. And sometimes a psychologist can be more critical of their client and have a more critical view than maybe AI does.

P: Yes, and plus we don't say why. We help the person to follow in order to find their why. Where, when did it happen, when did you learn this. And this is really transforming. This is the process, I'd say.

I: How do you think clients respond to AI being used in their treatment?

P: There is a difference between using AI in the sessions and AI doing sessions or doing the clinical work. But in the sessions, I think it would depend on age and the problem, the reason for the process. I think young people could accept it better, also because they tend to have more ease using these tools. It could be more difficult with other people. In general, when they use AI tools between sessions, they come with this information with some shyness, like, I checked on ChatGPT but he didn't say much. So I think if there is a way of using AI during the sessions as a tool, people could accept it because it would already be validated by the therapist. So the therapist would choose this tool to use, as we use Google Teams or other things. But first, I think it has to make sense to the therapist so we could introduce this tool like any other tool.

I: So you also mentioned that age might be a factor, where young people might be more inclined to be open to it and older people might be less. You also said that the type of problem might affect the willingness of the clients. So do you have any more information on that? Like what type of people with what type of problems would be more open to it and what people might not?

P: People with anxiety issues, isolating the problem, considering that it's a typical anxiety, for these isolated problems people can accept better. But for personality issues, it can be a big issue. Because in some personality issues there are people who don't trust enough, they pay only in cash because they don't want to use a card, so in some pathologies it may not be helpful.

I: So you think in general it might work, but then for some people there is some suspicion. So there are some pathologies that have suspicious tendencies, right. So maybe some people with psychotic symptoms or hallucinations or that kind of stuff, they might have some tendency to be very suspicious of people or of anyone in particular, so they might also be suspicious of AI listening in or AI being used and their data going somewhere.

P: I think we have to consider the ease of people using digital platforms in general. Because I think the main point would be the therapist considering this tool in their sessions. Because there are a lot of tools and we select the tools that we think might work with the person, and they still might not work together, even though you thought that they would. So it could be a tool like that. The therapist considers this tool because they are comfortable using it, and still it could not work for the person. So it has to start with the therapist.

I: Yeah, first of all the therapist has to have trust in this tool and believe it has potential to work, right. It might not work, but at least they think it could be a benefit for their client. And that's the first step. And then the second step will be from the therapist to introduce this to the client and discuss, okay, this is the reason why I think it might be helpful. We will try it out, and then you will see if the client approves or doesn't want to do that. But it's dependent on whether you

believe in the tool, and also on things like maybe young people are more willing to do it, maybe older people are less, and maybe some people with specific problems might not tend to agree with it because they're suspicious of other things. Do you think high comorbidity would also make it harder to introduce AI tools? So for example, if someone has anxiety and it's just anxiety, that's a bit more straightforward. But when you introduce comorbidity it becomes harder, because then it's like, oh, is this the depression or is this the anxiety or is it a mix of both? Like sleep could be depression or it could be anxiety or it could even be a manic episode or whatever, right?

P: Comorbidity, not always, but comorbidity can be related to personality structures.

I: Yeah, personality has a lot of comorbidity with a lot of things, right?

P: You never have a personality issue per se.

I: You always have something more.

P: With something, yeah. So if we consider what we said before about personality issues, there are comorbidities with that, so it could bring more resistance to use, but not only AI, any tools.

I: Professional identity, autonomy, and future conditions. Which aspects of your work should remain strictly human? And what would you need to feel confident about using AI safely in practice?

P: I would need to know for sure that there is security about the data and that the AI doesn't make decisions in the sense of diagnosis or suggestions of treatments in a way that it's the only one that works. It could be useful if the AI considers scientific information, like that.

I: And then the first part, which aspects of your work should remain strictly human?

P: The therapy, the therapeutic process.

I: So why do you think these aspects need to stay human?

P: Because we are social humans and we need to be in society. We need this belonging, it's a biological need, it's an evolutionary need. And in my view, we cannot have this with an AI tool.

I: Do you have anything else to add?

P: No, I think the interview is long because I talk too much.

I: No, in general it's nice to have a lot of information, so more talking is better.

P: If at some point you need answers to more questions because it's a qualitative research, I'm available to answer. But for now I have nothing more to say.

